# 4.11 Pericarditis and Cardiac Tamponade

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "The chest pain is sharp and worse when I lie down."<br>**Objective**<br>• Friction rub heard<br>• Leaning forward eases the pain | **ACTUAL**<br>**Acute pain** related to pericardial inflammation as evidenced by sharp positional chest pain and friction rub. | Pain 3/10 or less within 24 hours. | • Sit him up, leaning forward.<br>• Give the anti-inflammatory as prescribed.<br>• Check pain regularly. | Pain controlled: **goal met**. |
| **Subjective**<br>• "I feel breathless and faint."<br>**Objective**<br>• Low BP, distended neck veins, muffled heart sounds<br>• Pulsus paradoxus | **ACTUAL**<br>**Decreased cardiac output** related to compression of the heart by pericardial fluid as evidenced by hypotension, distended neck veins and muffled heart sounds. | BP back up after pericardiocentesis. | • Report signs of tamponade at once.<br>• Get ready for urgent pericardiocentesis.<br>• Continuous monitoring of vital signs and ECG. | BP restored: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Echo shows a pericardial effusion<br>• HR climbing | **RISK**<br>**Risk for decreased cardiac output** as evidenced by risk factors: increasing pericardial effusion. | Any sign of tamponade is picked up early and reported straight away. | • Check BP, HR and neck veins every 1–2 hours.<br>• Check for pulsus paradoxus the way you were taught.<br>• Keep the pericardiocentesis set ready.<br>• A falling BP, rising HR or muffled heart sounds must be reported at once. | No tamponade: **goal met**. |